When is it safe for Dad to drive?

Driving windows after Ativan, Dayvigo, and combinations · 2026-05-22

For Dad, Mom, Leah, David, Debbi, family team

THE SINGLE MOST IMPORTANT THING ON THIS PAGE

How Dad feels is not a reliable signal of whether he's safe to drive. Pomara 2015 showed that elderly patients on low-dose lorazepam — even 0.25 mg — had measurable psychomotor impairment *without subjectively feeling impaired*. "I feel fine" is not the test. The clock is the test, and on doses that span the day, family-observed steadiness and reaction time are the test.

QUICK REFERENCE — MINIMUM NO-DRIVE WINDOWS AFTER A DOSE

Scenario	Minimum no-drive	Safer window
Dayvigo (lemborexant) 5 mg at bedtime, no Ativan	~8 hours	Until 10 AM
Ativan 0.5 mg, alone	12 hours	24 hours
Ativan 1 mg, alone (the morning rescue dose)	24 hours	48 hours
Ativan + Dayvigo same evening	12 hours	Family check through afternoon
Ativan 0.5 mg AM after doxepin 6 mg + trazodone 50 mg HS (old stack)	12 hours	16 hours
Ativan 1 mg AM after doxepin 6 mg + trazodone 50 mg HS (old stack)	16 hours	No driving day-of
Any of the above + Depakote (if started)	Double the window	Skip driving day-of

Scenario timelines

No drive — drug at peak / clinically meaningful levels

Caution — family check required

Cleared — drug below impairment threshold

Scenario 1 — Dayvigo at bedtime only (the routine sleep stack) ~8 HR MINIMUM

Lemborexant 5 mg HS taken at 10 PM. No Ativan.

No drive | Family check | Cleared

10 PM dose | 6 AM | 10 AM | 10 PM

The published OE analyses covered Ativan-driving directly but didn't address Dayvigo-alone driving explicitly — the windows below are a conservative interpretation of the FDA Dayvigo label (warns of potential next-day impairment), lemborexant's 17-hour half-life, and Dad's specific risk profile. The Dayvigo SUNRISE driving studies in working-age adults found minimal impairment the morning after a 5 mg dose. For Dad, given prodromal Lewy-spectrum markers and gait abnormality, **prefer not to drive before 10 AM** the morning after a bedtime dose. Family-observed steadiness check before driving is the floor.

Scenario 2 — Ativan 0.5 mg, used during the day 12 HR MINIMUM, 24 HR SAFER

Lorazepam 0.5 mg taken at noon. No other CNS-depressant additions today.

No drive | Family check | Cleared

noon dose | midnight | noon next day | +4 hr

A 0.5 mg dose produces measurable psychomotor impairment for at least 12 hours in elderly patients per Pomara 2015. **Don't drive on the same calendar day as a 0.5 mg dose.** The next morning, if Dad needs to drive, a family member should check steadiness, gait, and reaction time first.

Scenario 3 — Ativan 1 mg (the morning-rescue size dose) 24 HR MINIMUM, 48 HR SAFER

Lorazepam 1 mg taken at 7 AM, e.g., to abort an acute anxiety episode like the one on 5/21 morning.

No drive | Family check | Cleared

7 AM dose | +24 hr | +48 hr

The 1 mg dose used on 5/21 morning is twice the half-life-clearance burden of the 0.5 mg scenario. The FDA injectable label specifies 24–48 hours no-driving after a 1 mg dose; Seppälä 1976 found measurable impairment at 12+ hours from a 2.5 mg dose in young healthy volunteers. For Dad, **plan for two no-drive days** after any 1 mg dose. If Dad needs to drive on day 2, family check first.

Scenario 4 — Ativan in the evening + Dayvigo at bedtime

12 HR MIN; CAUTION THROUGH AFTERNOON

Lorazepam 0.5 mg at 9 PM (pre-bed anxiety rescue) + lemborexant 5 mg at 10 PM. Additive CNS depression overnight.

No drive	Family check	Cleared
9 PM dose	9 AM next day	5 PM

OE published guidance: **12 hours minimum after the Ativan dose**, with the caveat that additive CNS depression and lemborexant's 17–19 hour half-life mean residual effects overlap through the next day. The lemborexant FDA label specifically warns of increased daytime impairment when combined with benzodiazepines, especially in elderly patients. **Don't drive in the morning**; if Dad must drive in the afternoon, family steadiness check first. Default to late-afternoon or skip-the-day if function isn't time-pressed.

Scenario 5 — Doxepin 6 mg + trazodone 50 mg HS (the old stack, if Dad is still on it)

8–12 HR MIN; FAMILY CHECK

Doxepin 6 mg HS + trazodone 50 mg HS taken at 10 PM. No Ativan today. No lemborexant.

No drive	Family check	Cleared	
10 PM dose	8 AM	2 PM	10 PM

The complication on this stack is that **doxepin's active metabolite (nordoxepin) has a 31-hour half-life** and accumulates over consecutive nights to steady state by ~5 days. Trazodone clears mostly by morning (elderly half-life ~8 hours, ~35% remaining at 12 hours) — but the doxepin/nordoxepin component lingers and the residual sedation grows night by night during the first week. **Family-observed steadiness check before any morning driving is the operative tool**, especially after the first 3–5 nights of dosing. The FDA-labeled elderly starting dose for doxepin is 3 mg, not 6 mg — worth raising with the prescribing team given the residual-sedation question (see callout below).

Scenario 6 — Ativan AM rescue on top of the old stack from the night before

12–16 HR MIN; LIKELY NO DRIVE DAY-OF

Doxepin 6 mg + trazodone 50 mg HS taken last night (10 PM). Then Ativan 0.5–1 mg taken AM today as PRN rescue.

No drive	Family check	Cleared
8 AM Ativan dose	midnight	8 AM next day

The morning Ativan layers GABA-A sedation on top of residual nordoxepin (31-hr half-life), residual trazodone (~35% remaining at 12 hours), and any accumulated steady-state from prior nights. Three distinct sedative mechanisms overlap. **For 0.5 mg Ativan: 12 hr minimum, 16 hr safer. For 1 mg Ativan: 16 hr minimum, 24 hr safer — effectively no driving that day**, and probably no driving the next morning either. Plus the dominant safety concern on this combination is fall risk, not just driving (see callout below).

DOXEPIN + TRAZODONE CONTEXT — IMPORTANT IF ON THE OLD STACK

Nordoxepin (doxepin's active metabolite) has a 31-hour half-life. This is the longest-acting component in any sleep stack containing doxepin. It accumulates over consecutive nights, reaching steady state at ~5 days. After a week of dosing, residual next-morning sedation is meaningfully higher than after the first night. This is why the family-steadiness check matters more — and gets harder to pass — the longer the regimen runs.

The FDA-labeled doxepin starting dose in elderly (≥65) is 3 mg, not 6 mg. Dad is on the maximum labeled dose for any age group and double the recommended elderly starting dose. If the prescribing team isn't already aware, raising this with them — and asking whether 3 mg would give comparable sleep with less residual sedation — is worth doing.

Trazodone is not a low-risk fall agent in elderly. Bronskill 2018 found trazodone's fall-injury rate (5.7% at 90 days in nursing homes) was statistically equivalent to benzodiazepines (6.0%; HR 0.94). Rivasi 2025 found trazodone users ≥75 had 58.3% syncope/falls vs. 21.2% in non-users (p=0.001). Combined with Dad's baseline BP 112/58 and sinus bradycardia (HR 49–53), trazodone is a meaningful fall-risk contributor on its own.

Antidepressant + benzodiazepine combinations independently raise fall risk. Wang 2025: moderate-dose antidepressant + low-dose benzodiazepine = HR 1.71 for injurious falls; higher doses = HR 1.96. Dad on duloxetine + Ativan PRN, with doxepin/trazodone added at night, is in the higher-risk band of this finding.

IF DEPAKOTE (DIVALPROEX) GETS STARTED — READ THIS CAREFULLY

Valproate **doubles Ativan's effective blood level** by slowing its clearance by ~40%. The FDA label mandates reducing the Ativan dose by 50% when valproate is on board, because a 0.5 mg dose becomes pharmacokinetically equivalent to a 1 mg dose.

If Depakote ends up on board, double every window above. A 0.5 mg Ativan dose now requires the 24-hour window. A 1 mg Ativan dose requires 48 hours minimum. The Dayvigo + Ativan combination becomes a clear "no driving day-of" call.

Family check before driving — the steadiness test

IF DAD IS IN ANY "AMBER / FAMILY CHECK" WINDOW ABOVE, RUN THROUGH THESE BEFORE HE DRIVES

1. **Stand from a seated position without using arms for push-off.** Slowed? Off-balance? Don't drive yet.
2. **Walk 20 feet in a straight line, turn, walk back.** Watch for hesitation, drift, or shortened steps relative to his baseline.
3. **Stand still with feet together, eyes closed, for 10 seconds.** Sway? Step-out for balance? Don't drive.
4. **Brief reaction-time check:** drop a pen, ask Dad to catch it. Or stand at arm's length and ask him to tap your hand as soon as you raise it. Slowed reaction relative to baseline? Don't drive.
5. **Quick conversation:** ask Dad to tell you the date, his next appointment, and what he had for breakfast. Word-finding hesitation, confusion, or unusual flatness of voice? Don't drive.
6. **Ask Dad how he feels.** If he says "fine" but any of items 1–5 looked off, the assessment is "don't drive," not "fine." Pomara 2015: subjective feel is unreliable.

Other points worth knowing

- **Alcohol changes everything.** Even one drink within the no-drive window or the family-check window above resets the timer to "no drive day-of." Lemborexant + alcohol is specifically warned against on the FDA label.
- **Time of dose matters.** A morning Ativan dose extends the no-drive window through the entire day; a bedtime dose mostly burns through overnight. If Ativan is needed, prefer evening dosing when possible — but don't game timing to enable driving.
- **Cumulative use changes the calculus.** The windows above are for occasional PRN use. If Ativan use becomes near-daily, the windows extend further and family steadiness checks become the operative tool day-to-day.
- **The bigger picture.** If Dad's medication regimen is producing frequent no-drive days that limit his independence, that's information worth bringing to the U-M conversation. The goal isn't to maximize the windows — it's to get to a stable regimen where the driving question isn't a daily problem.

Windows above reflect OpenEvidence-summarized published guidance (FDA labels for oral and injectable lorazepam, doxepin, trazodone, lemborexant; Pomara 2015 elderly psychomotor study; Seppälä 1976 driving impairment study; Greenblatt 1987 trazodone PK in elderly; Bayer 1983 trazodone in elderly; Rivasi 2025 trazodone-syncope; Seppala 2018 psychotropic fall-risk meta-analysis; Bronskill 2018 trazodone-vs-benzodiazepine falls; Wang 2025 antidepressant-benzodiazepine fall HR; Johnell 2017 psychotropic polypharmacy and fall hospitalization) interpreted for Dad's specific profile (age 76, sinus bradycardia, prodromal Lewy-spectrum markers including gait abnormality and balance problems, concurrent duloxetine and possible divalproex). The Hemmelgarn 1997 cohort (n=224,734 elderly drivers, ages 67–84) was somewhat reassuring at the population level for short-half-life benzodiazepines including lorazepam (rate ratio 1.04, not statistically significant), in contrast to long-half-life benzodiazepines (rate ratio 1.45); however, that study did not stratify by concurrent CNS depressants or neurodegenerative disease markers, so for Dad's specific profile the more conservative individual-patient evidence governs. U-M and Dr. Maxner may give updated guidance once they take over psychiatric care.